Plan: PLAN-A | Drugs: Humira, Adalimumab | Version: 2026-01 | Effective: 2026-01-01

PLAN-A Pharmacy Policy: TNF Inhibitor Biologics

Overview

This policy explains how PLAN-A reviews requests for tumor necrosis factor (TNF) inhibitor biologic medications. Its purpose is to give members, prescribers, and reviewers one consistent reference for what the plan requires before these costly specialty drugs are approved, so that decisions are predictable and timely.

The policy covers Humira (adalimumab), a TNF inhibitor. Prior Authorization (PA) is required, and step therapy applies.

Reviews are performed by plan pharmacists and clinical reviewers who apply the criteria below to the information submitted. Reviewers may contact the prescriber for clarification. Standard requests are normally decided within several business days of receiving a complete submission, and expedited review is available when a delay could seriously affect the member's health.

Decisions are sent to the prescriber and the member in writing. A denial states the reason and the criteria applied, and it may be appealed through the plan's standard appeals process. Appeals are reviewed by a clinician who took no part in the first decision. The version of this policy in effect on the date a request is received is the one applied to that request.

Coverage Criteria

A request is considered for approval when all of the following conditions are met.

- The member is an adult with moderately to severely active rheumatoid arthritis.

- The medication is prescribed by or in consultation with a rheumatologist.

- The medication is not covered in combination with another biologic DMARD.

Meeting these criteria does not by itself guarantee approval. Reviewers also confirm that the member is actively enrolled on the date of the request, that the prescription is valid, and that the information supplied agrees with the clinical notes. A request that conflicts with the documented treatment plan may be returned to the prescriber for correction before a decision is made.

Members who believe their situation does not fit the criteria may ask the prescriber to submit an exception request. An exception request should explain why the standard criteria do not suit the member's circumstances and is decided case by case by a plan clinician.

Required Documentation

Each PA request must include the items below. A complete first submission is the fastest route to a decision, so prescriber offices should attach everything at the outset.

- Clinical notes documenting the rheumatoid arthritis diagnosis and current disease activity.

- A rheumatology consultation note.

- Tuberculosis screening result within the last 12 months.

- Hepatitis B screening result.

- Documentation of the methotrexate trial (start and end dates, dose, outcome) or a documented contraindication.

- Prescriber name and NPI.

Incomplete requests are returned as "documentation incomplete". The return notice lists each missing item so the prescriber can supply it without resubmitting the whole request, and the review clock restarts when the missing items arrive.

Clinical notes should be recent enough to reflect the member's present condition and should be signed or otherwise authenticated by the treating clinician. Disease activity should be described with the measures the practice normally uses, such as joint counts or a composite activity score, so that the reviewer can judge severity and later compare progress.

Screening results should show the collection date, the reported finding, and the name of the performing laboratory. Legible scanned or faxed copies are acceptable. Prescriber identification is used to confirm that the request comes from a licensed clinician who participates with the plan.

Step Therapy

The member must have tried methotrexate for at least 3 months at an adequate dose without sufficient response, or have a documented contraindication or intolerance, before Humira is approved.

Methotrexate use is verified from pharmacy claims history. If claims do not show methotrexate, the prescriber must supply the documentation. This may include prior pharmacy records, clinic medication lists, or notes written at the time of the trial.

When a contraindication or intolerance is the reason methotrexate was not used or was stopped, the notes should name the reason and the date it was identified. Reviewers assess whether the documented trial or reason is reasonable for the member's situation and record the outcome of that review in the case file.

Quantity Limits

The plan limits how much of the medication may be dispensed in a single fill period so that supplies match the usual dosing schedule.

- 2 pens per 28 days for maintenance dosing.

- An initial loading dose may be approved once with documentation.

A request for the initial loading dose should state the planned dosing schedule and the date therapy is expected to begin. Once the loading dose has been approved and dispensed, later fills follow the maintenance limit.

Pharmacies that receive a quantity rejection should contact the prescriber, who can submit supporting documentation through the same channel used for the original PA. Reviewers evaluate such requests individually and may ask for further clinical detail before deciding.

Lost, stolen, or damaged supplies are handled through a separate review and are not governed by these limits. Members should speak with their pharmacy or the plan's member services line for help with those situations.

Renewal

Approvals last 12 months from the approval date. The approval notice states the exact end date, and prescribers are encouraged to start the renewal well before that date so that treatment is not interrupted.

Renewal needs clinical notes showing a documented response to treatment. The notes should compare the member's current disease activity with the findings recorded at the start of therapy, using the same measures where possible. A renewal is reviewed against the criteria in effect on the date it is received, which may differ from those that applied to the original approval.

A renewal request that arrives after an approval has ended is treated as a new request, and the full documentation described earlier in this policy applies. A gap in coverage may occur while the new request is under review. Prescribers who are unsure whether a request counts as a renewal or a new request may ask the plan's pharmacy help desk before submitting.

This is a fictional policy created for software demonstration only.